

**STATE OF VERMONT
SECRETARY OF STATE
OFFICE OF PROFESSIONAL REGULATION
VERMONT BOARD OF ALLIED MENTAL HEALTH PRACTITIONERS**

In re: NATHAN KOKERNOT	}	
License No. 068.0135138	}	Docket No. 2026-7
	}	

Participating Board Members:

Marjorie Trombly, LCMHC, Vice Chair
Jessyca Nelle, LCMHC
Caitlyn Pippin, Public Member
Dana Poverman, *ad hoc*, LCMHC
Marilyn Turcotte, *ad hoc*, LCMHC

Appearances:

Prosecutor: Zoe Newman, Esq.
Respondent: Appeared and represented himself

Presiding Officer: George K. Belcher

Exhibits:

State Exhibit 1: Email from M. Desjardins to Respondent and Respondent's Response

**Summary Suspension Order
FINDINGS OF FACT, CONCLUSIONS OF LAW,
AND ORDER**

This matter came before the Board of Allied Mental Health Practitioners on a Request for a Summary Suspension. The hearing was held on February 19, 2026. The Prosecutor for the State, the Respondent, the witness and the Board members all participated by telephone, in person, or via electronic means pursuant to the Office of Professional Regulation Rules.

Findings of Fact

Based on the evidence presented at the hearing and the arguments of counsel, the Board finds as follows:

1. Respondent is licensed as a Licensed Clinical Mental Health Counselor (LCMHC) in the State of Vermont.
2. The Respondent is subject to the regulatory authority of this Board. 3 V.S.A. §§ 129, 129a, 26 V.S.A. Chapter 65, the Administrative Rules of the Board of Allied Mental Health, and the Rules of the Office of Professional Regulation. 3 V.S.A. § 814(c) permits the Board to summarily suspend a license if it finds that the public health, safety, or welfare imperatively requires emergency action, and incorporates a finding to that effect in its order.

3. The prosecutor has filed a “Request for Summary Suspension.”
4. The Respondent participated in the hearing of this case. There were no objections as to notice.
5. In December of 2025 the Office of Professional Regulation received a complaint from a client (hereinafter referred to as “JB”) regarding her treatment from the Respondent. The complaint included claims: (1) that the client had been diagnosed by two psychiatrists with “mood disorder” but that the Respondent told her that he does not believe in mood disorders; (2) that the Respondent thought the client, in his opinion, did not need therapy; (3) the Respondent had felt that way for a few years; (4) the Respondent tried to persuade the client not to end her treating relationship with the Respondent; (5) the Respondent told the client that he had developed “feelings” for the client and had held those feelings for about a year; he told her that the feelings were “20% romantic and 80% tender.” The client felt that the Respondent had violated ethical boundaries and the client questioned the guidance the Respondent had given to her during the past year.
6. The Respondent filed an email response to the client’s complaint which stated in part, “I do not dispute any of the things that she reports I said in that report.” (See State Exhibit 1)
7. According to the investigator, M. Desjardins, the client told him that the Respondent falsely told her that he did not have to wait six months to have a personal relationship with the client after the treating relationship ended.
8. The Respondent’s testimony did not contradict the essential facts of the client’s complaint. In his testimony he said that: (1) he had not had a supervisor to his practice since he was licensed; (2) he did not regularly keep treatment notes or records of his treatment sessions and when he did keep notes they were “not very good”; (3) he did not follow professional ethics associated with the profession of License Mental Health Counselling unless those ethics coincided with his own ethics; (4) he diagnoses almost all of his clients with “adjustment disorder.” His diagnosis of “adjustment disorder” is given to clients, not because of any diagnostic tool or process, but simply because life creates situations which require adjustments for all of us, and “adjustment disorder” is a diagnosis which allows him to be paid by insurance.
9. The Respondent admitted that he tried to persuade the client, “JB”, that she did not have a mood disorder despite that diagnosis having been given to her by two qualified providers. The Respondent believed that the diagnosis was not valid because “we are all equally broken.”
10. The Respondent provided the client “JB” and his other clients with no treatment plan.
11. The Respondent does not, and has not, discussed his diagnoses of “adjustment disorder” with his clients unless they ask about it, despite the fact that all, or almost all, of his clients receive that diagnosis and that diagnosis is reported to insurers.
12. The Respondent understood the concerns expressed by client “JB” and her feelings that he violated professional boundaries.
13. The Respondent did not show remorse regarding his breach of professional ethics or boundaries. He showed no insight into the effects of his failure to keep records or to make diagnoses without any clinical basis or evaluation. He testified that if his license were not removed, he would continue to practice in the same manner as he has. He showed no respect for the ethical standards of the profession and elevated his own “ethics” above Vermont’s legal and professional requirements.

14. At one point during the testimony of the Respondent, he digressed into a discussion asserting that each person's reality is solely within their minds based on history and experiences, as opposed to what is real. He testified that mathematicians are close to proving this fact. He stated that "space and time are doomed" and that he can make no judgments about others.

Conclusions of Law

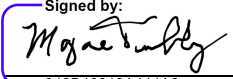
1. The evidence established that the Respondent violated: 26 VSA Sec. 3271(a)(3) [conduct which evidences unfitness to practice mental health counseling]; 26 VSA Sec. 3271(a)(6) [Entering into an additional relationship with a client, supervisee, research participant, or student that might impair the licensed clinical mental health counselor's objectivity or otherwise interfere with the clinical mental health counselor's professional obligations]; 3 VSA Sec. 129a(a)(19) [Willful misrepresentation in treatment or therapies]; 3 VSA Sec. 129a(a)(28) [Engaging in conduct of a character likely to deceive, defraud, or harm the public]; 3 VSA Sec. 129a(b)(1) [Failure to practice competently by reason of any cause including performance of unsafe or unacceptable patient or client case].
2. The evidence presented proved facts which imperatively require emergency action for public health, safety, and welfare. Specifically, summary suspension is necessary to prevent patient harm, and to protect the public.
3. The Findings of Fact and Conclusions of Law in this Summary Suspension hearing are for purposes of deciding whether **at this time** there is an imperative need to take emergency action. 3 V.S.A. § 814(c). The Findings of Fact and Conclusions of Law herein are for purposes of this Order only.
4. The Prosecutor's Request for a Summary Suspension is supported by the evidence presented, and the Findings of Fact above support Summary Suspension of Respondent's license. There is an imperative need to take emergency action. 3 V.S.A. § 814(c).
5. Vermont law requires that unprofessional charges be filed promptly with Respondent being afforded a prompt hearing. At any merits hearing in this matter, the prosecutor will bear the burden of proving unprofessional conduct. The Findings and Conclusions in this matter will not absolve the prosecution or Respondent from producing or challenging relevant evidence at a merits hearing. The Findings of Fact and Conclusions of Law at a contested hearing will be based exclusively on the evidence admitted at that hearing.

Order

The Request for Summary Suspension is **Granted**. Respondent's license is **Summarily Suspended** and shall not be subject to renewal or reinstatement during the period of Summary Suspension without Board approval pending proceedings. These proceedings shall be promptly instituted and determined. 3 V.S.A. § 814(c).

SO ORDERED.

Vermont Board of Allied Mental Health Practitioners

By: 
Signed by:
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 Marjorie Trombly, Vice Chair

Date: 2/20/2026, 2026

OFFICE OF PROFESSIONAL REGULATION

DATE OF ENTRY: 2/20/2026

APPEAL RIGHTS

This is a final administrative determination by the Vermont Board of Allied Mental Health Practitioners or Director of the Office of Professional Regulation. A party aggrieved by a final decision of a board or hearing authority may appeal this decision by filing a written Notice of Appeal with the Director of the Office of Professional Regulation, Vermont Secretary of State, 89 Main Street, Fl. 3, Montpelier, VT 05620-3402 within 30 days of the entry of this order. If an appeal is filed, the Director of the Office of Professional Regulation shall assign the case to an appellate officer. The review shall be conducted on the basis of the record created before the board. In cases of alleged irregularities in procedure before the board, not shown in the record, proof on that issue may be taken by the appellate officer. 3 V.S.A. §§ 129(d) and 130a.